

First-line therapy for peritoneal metastatic gastric cancer

Dr. med. Laura Predel, University Hospital Leipzig

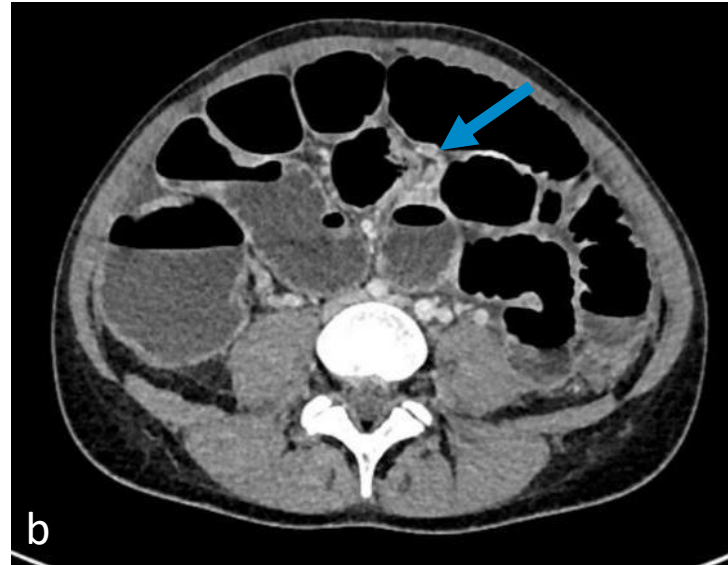
March 5th 2026



- ☞ **Symptoms:** Since July 2025, weight loss of 4 kg (9% of body weight), fatigue, upper abdominal pain
- ☞ **Diagnoses:** Iron deficiency anemia, suspected Hashimoto's thyroiditis and gastritis
- ☞ **January 4, 2026, Emergency Department:** Diarrhea, vomiting, BMI 16.6 kg/m²
 - **Esophagogastroduodenoscopy:** ulcers in gastric fundus, pyloric stenosis
 - **Colonoscopy:** impassable stenosis at 40 cm

Computed tomography of the abdomen

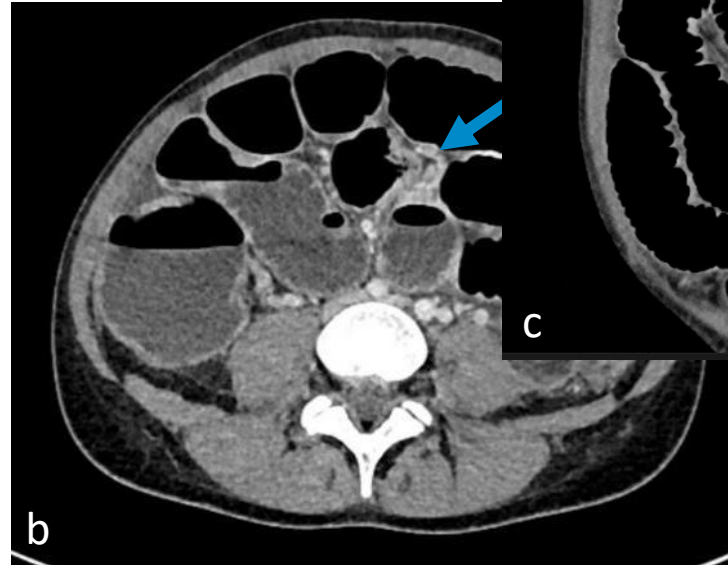
- Multiple long-segment stenosing wall thickenings
 - Small and large bowel ileus
 - Ascites
- Interpreted as a possible diagnosis of Crohn's disease



Illustrations:
Stenosis of the
pylorus (a) and
jejunum (b)

Computed tomography of the abdomen

- Multiple long-segment stenosing wall thickenings
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Illustrations:
Stenosis of the
pylorus (a), jejunum
(b) and colon
transversum (c)

Histology of gastric mucosa:

- Poorly differentiated non-cohesive diffuse adenocarcinoma

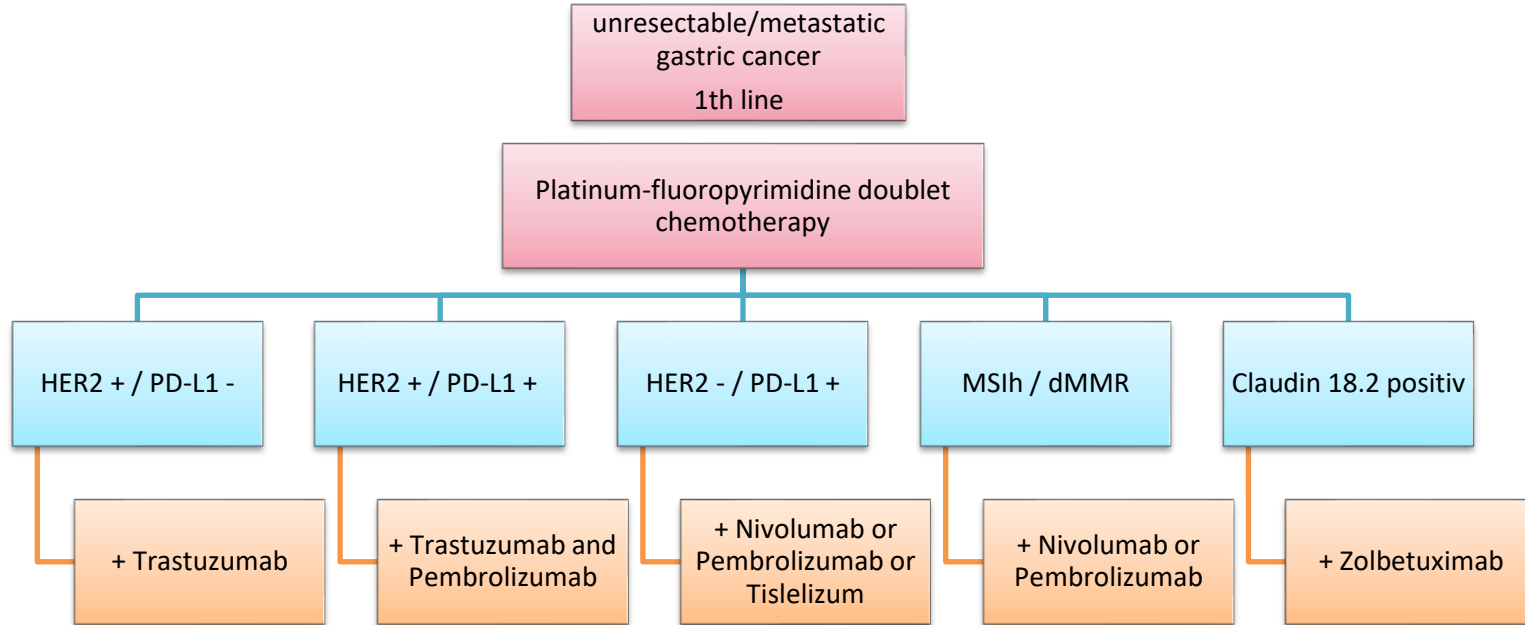
TNM: cT3 cN+ cM1(PER), UICC stage IV

Immunohistochemistry:

- pMMR (MLH 1, MSH2, MSH6, PMS2)
- Her2 negativ (IHC Score 0)
- PD-L1 CPS 6
- Claudin 18.2 positiv (ca. 95%, IHC Score 3+)

Female, 26 yrs

cT3 cN+ cM1(PER), pMMR, Her2-, PD-L1 CPS 6, Claudin 18.2 95%

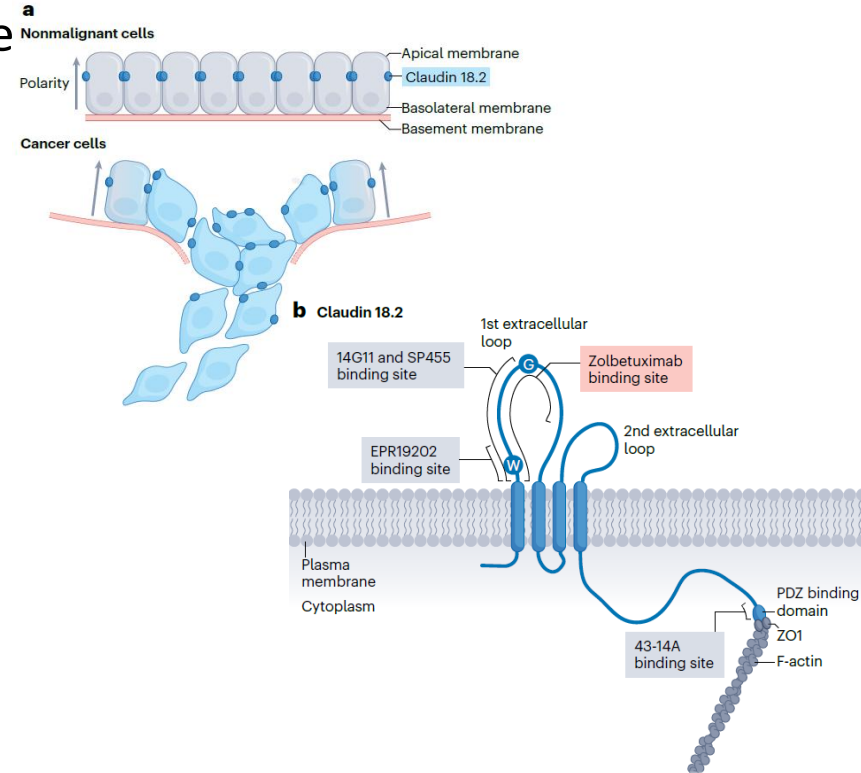


Modified from www.onkopedia.com/de/onkopedia/guidelines/magenkarzinom/ (last access 26.01.26)

🔵 **Claudin 18.2:** Tight-junction protein of the gastric mucosa

🔵 **Zolbetuximab:** Chimeric monoclonal antibody against claudin 18.2

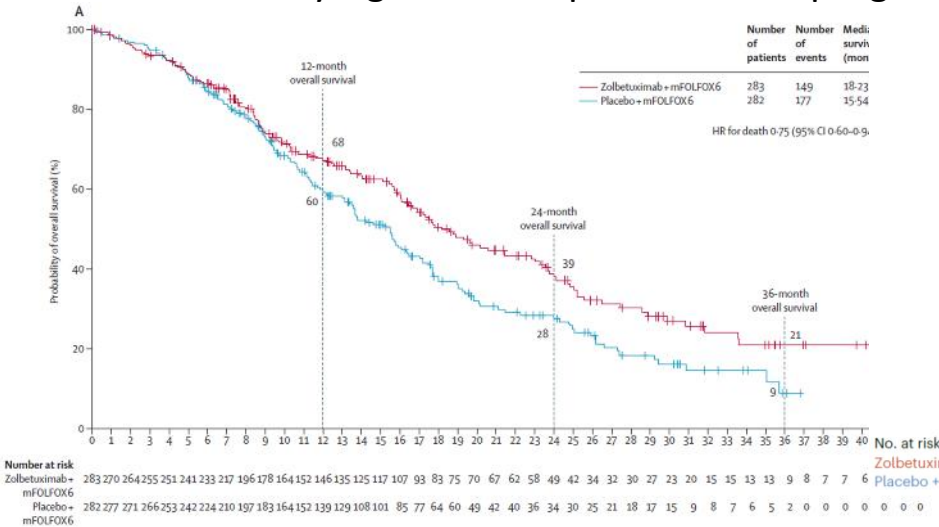
🔵 Very common ($\geq 1:10$): Nausea, vomiting, decreased appetite



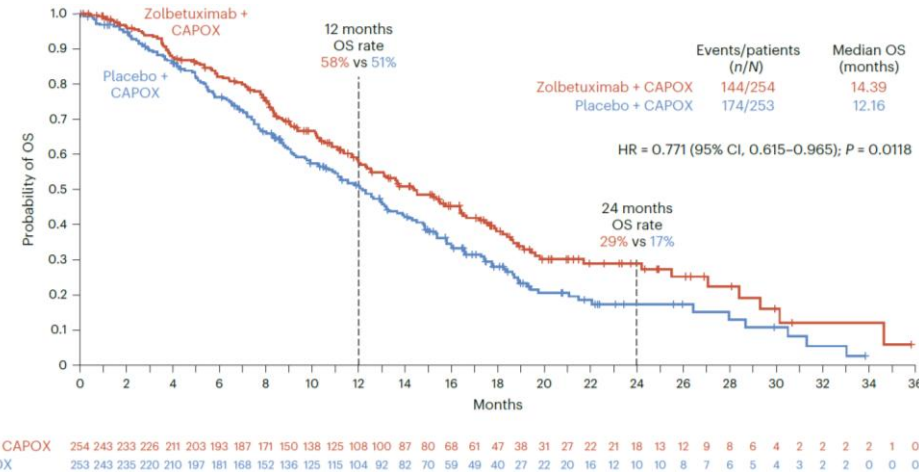
Nakayama I et al. *Nat Rev Clin Oncol.* 2024 May;21(5):354-369

SPOTLIGHT

- Randomized, controlled phase III trial, mFOLFOX6 or CAPOX +/- zolbetuximab
- Statistically significant improvement in progression-free and overall survival



GLOW-trial



Shitara K, et al. Zolbetuximab plus mFOLFOX6 in patients with CLDN18.2-positive, HER2-negative, untreated, locally advanced unresectable or metastatic gastric or gastro-oesophageal junction adenocarcinoma (SPOTLIGHT): a multicentre, randomised, double-blind, phase 3 trial *Lancet*. 2023;401(10389):1655-1668.

Shah M, et al. Zolbetuximab plus CAPOX in CLDN18.2-positive gastric or gastroesophageal junction adenocarcinoma: the randomized, phase 3 GLOW trial *Nature Med*. 2023 Aug;29(8):2133-2141

Female, 26 yrs

cT3 cN+ cM1(PER), pMMR, Her2-, PD-L1 CPS 6, Claudin 18.2 95%

📅 January 14, 2026: **C1 mFOLFOX6**

📅 January 16, 2026: Dilation of gastric outlet obstruction, stent placement

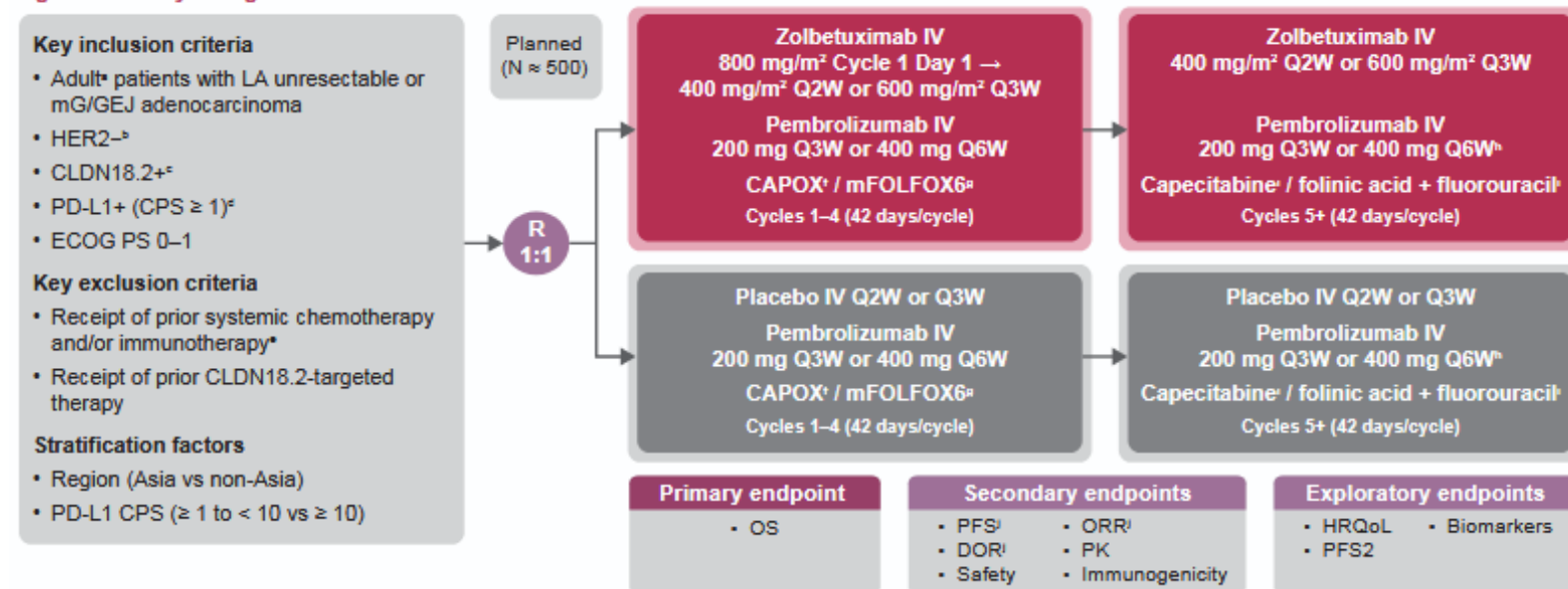
📅 January 19, 2026: Consultation at **University Hospital Leipzig** regarding pain exacerbation, second opinion

- Enrollment in the LUCERNA trial?
- Zolbetuximab vs. placebo plus pembrolizumab and chemotherapy (CAPOX or mFOLFOX6)

- Randomized, controlled phase III trial: First-line treatment of Claudin18.2-positive, PD-L1-positive and HER2-negative locally advanced or metastatic gastric or GEJ adenocarcinomas

Kohei Shitara et al., Kashiwa City, Japan

Figure 1. Study design



→ not eligible due to subileus

Question

F, 26 y.: cT3 cN+ cM1(PER), pMMR, Her2-, PD-L1 CPS 6, Claudin 18.2 95%



Universitätsklinikum
Leipzig

Medizin ist unsere Berufung.

Which systemic therapy would you suggest?

- a) Platinum-fluoropyrimidine doublet chemotherapy alone
- b) Doublet chemotherapy in addition of Nivolumab
- c) Doublet chemotherapy in addition of Claudin 18.2 targeted therapy
- d) Pembrolizumab monotherapy
- e) None of the above

Tumor board recommendation Univesital Hospital Leipzig

- LUZERNA not possible due to subileus
- Continuation of systemic therapy with mFOLFOX6 in addition of zolbetuximab
- Genetics testing – CDH1 germline mutation?

Question

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Medizin ist unsere Berufung.

Which surgical questions should be addressed?

- a) Resection of the primary tumor including total gastrectomy?
- b) Cytoreductive surgery with intraperitoneal chemotherapy?
- c) Diagnostic laparoscopy to confirm peritoneal metastasis?
- d) Prophylactic colostomy?

- ☞ Consider malignant diseases as a differential diagnosis, even in young patients.
- ☞ Stenosis and ileus symptoms pose particular challenges in GI tumors.
- ☞ Zolbetuximab in patients with claudin 18.2 expression $\geq 75\%$ (IHC 2+, 3+) in combination with systemic therapy prolongs PFS and OS in unresectable/metastatic gastric cancer.
- ☞ Management of gastrointestinal side effects with zolbetuximab is essential.

Thank you for your attention

